



Third Stage of Labour and Birth Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Health Service; Kalamunda Hospital	Obstetrics	Midwives, Nursing, GP Obstetrics

2. Purpose

The purpose of this guideline is to ensure that primary care givers attending birth, and women in their care, are fully and accurately informed of the principles and risks associated with options for third stage management and supported in their choice.

3. Background

The third stage commences with the birth of the baby and is considered completed at birth of the placenta and membranes. Management of third stage is commonly classified into active or physiological in relation to whether specific elements of care are routinely included. It can be actively or physiologically managed depending on the woman's wishes and assessment of pre-existing risk factors. Women should be counselled in advance on the need to use a uterotonic to prevent postpartum haemorrhage (PPH), the available uterotonic options, and their possible side-effects.

4. Definitions

4.1 Active Management of third stage

- Active management of the third stage of labour consists of interventions designed to facilitate the delivery of the placenta by increasing uterine contractions and to prevent primary postpartum haemorrhage (PPH). This usually includes administration of uterotonic agents, controlled cord traction after birth of the placenta.
- Active management is recommended to shorten the third stage and reduce the risk of maternal postpartum haemorrhage and the need for blood transfusion.
- To maximize efficacy, uterotonics are best given immediately (preferably within one minute) after the birth of the baby preferably at birth of anterior shoulder (or second baby in twin pregnancy). Administration of a uterotonic for prevention of PPH need not impede the delaying of cord clamping.

4.2 Physiological management (also referred to as expectant management)

- Physiological third stage of labour is achieved when the delivery of the placenta and membranes occur spontaneously, and haemostasis is achieved physiologically.
- Placental separation and expulsion without intervention should be reserved for women who are low risk and have been assessed and counselled about both modes of management and

completed a Non-Standard Management Plan with their primary care provider as per [Non-Standard Management Plans: Obstetrics and Gynaecology Procedures](#)

- Primary care givers must ensure that the woman is aware that any change in clinical picture, that falls outside of normal and if the risk of PPH increases, active management is then appropriate and will be advised.
- If the woman no longer fits the criteria of low risk during her labour and is fully informed of such, yet wishes to have a physiological third stage, inform the GP/Obstetrician and complete a non-standard management sticker in the woman's notes [Non-Standard Management Plans: Obstetrics and Gynaecology Procedures](#)

4.3 Delayed cord clamping

The World Health Organisation (WHO) recommends cord clamping one to three minutes after birth while initiating simultaneous essential newborn care.

- Early cord clamping (less than one minute after birth) is not recommended unless the neonate is asphyxiated and needs to be moved immediately for resuscitation.
- Delaying cord clamping, usually 1-3 minutes, results in higher haemoglobin and haematocrit values in the newborn and, possibly, lower levels of early childhood anaemia and greater iron stores. These effects are probably more profound in preterm infants and may result in fewer transfusions in the neonatal period and lower rates of neonatal intraventricular haemorrhage and sepsis.

5. Third stage progression

5.1 Signs of placental separation:

The most reliable sign of placental separation is the lengthening of the umbilical cord as the placenta separates and is pushed into the lower uterine segment.

- Placing a clamp on the cord at 1-3 minutes close to the perineum post birth and following oxytocic administration, makes it easier to visualise this lengthening.
- The uterus takes on a more globular shape and rises in the abdomen ('ballotable'). There should be no manipulation or massage of the uterus.
- A gush of blood usually occurs as the placenta separates from the uterine wall and descends to the lower uterine segment. This is not a reliable indicator of complete separation, as this is also associated with partial separation.

5.2 Controlled cord Traction (CCT):

- Following signs of separation place non-dominant hand just above the level of the symphysis pubis, applying gentle counter pressure in an upward direction, thus stabilising the uterus during CCT. This is sometimes referred to as 'guarding the uterus'. Do not manipulate the uterus.
- With the clamp in place approximately 10 cm from introitus, place fingers of dominant hand around clamp and place sustained downward traction on the cord (CCT) following the direction of the birth canal until the placenta appears at the vaginal entrance.
- The clamp may need to be moved up the cord to maintain consistent traction and to prevent snapping of cord under increased tension.
- If the placenta does not descend during 20 - 30 seconds of CCT or if there is resistance to CCT, do not continue to pull continuously on the cord.

- Release pressure on cord, await a contraction and repeat CCT. Provided there is no excessive bleeding or suspected separation of the cord from the placenta continued attempts at CCT may continue.

5.3 Birthing the placenta and membranes

- As the placenta emerges, release cord traction and counter pressure on the uterus just above the level of the symphysis pubis.
- Using non-dominant hand, cup the placenta as it emerges, and the dominant hand releases the cord after the bulk of the placenta has birthed.
- Hold the placenta in two hands and gently turn it to help release the membranes without tearing them. While pulling, use an upward and downward or a twisting movement to ease the remaining membranes slowly out of the vagina to complete third stage. If the membranes start to tear use a sponge forceps to clamp and twist remaining membranes into a remove any pieces of membrane that are present.
- Document time of birth of placenta and membranes.
- Immediately following removal of the placenta and membranes, massage the fundus of the uterus to make sure it is well contracted.

6. Active management of the third stage

6.1 Uterotonics

6.1.1 Minimising risk of inadvertent uterotonic administration

- Collect and store kidney dish and only take third stage oxytocics into the birthing room when the woman has commenced the active phase of the second stage of labour
- The practitioner who is responsible for administration of the medication checks the contents and expiry date of the vial with another practitioner prior to administration.
- After drawing up the contents into a syringe, label with an 'Oxytocin' sticker and keep the checked drug vial and syringe containing the uterotonic in a dish on the bottom of the delivery trolley (so it is away from the neonatal resuscitation cot to minimise inadvertent administration to the neonate).

6.1.2 Timing of administration

- Recommended administration of a prophylactic intramuscular oxytocic agent to mother is with delivery of the anterior shoulder or within 1 minute of delivery.
- Wait at least 1-3 minutes after birth or for cord pulsation to cease and then clamp and cut cord.

Do not delay clamping if the neonate requires any resuscitation.

- Can be administered before or after the cord is clamped and cut.
- Administration before cord clamping is unlikely to impact on placental transfusion to baby.

Considerations for active management of third stage:

- Breech presentations – ensure administration of oxytocic occurs after the birth is completed.

- Multiple births- ensure that oxytocic administration only occurs after the birth of the last foetus (e.g. second twin) otherwise excessive uterine contraction can result in death of the subsequent foetus (es).
- If possibility of an undiagnosed second twin (e.g. no ultrasound in pregnancy), exercise caution before administering oxytocic.

6.1.3 Administration of uterotonics

Low-risk patient:

- Oxytocin 10 IU
- 10 units administered via intramuscular injection (IM).
- Has minimal side effects and used for women with a low-risk pregnancy having an actively managed third stage.

High-risk patient:

- Syntometrine® 1 mL IM (pre-mixed Ergometrine 0.5 mg + Oxytocin 5 units) unless contraindicated.
- Administered via Intramuscular Injection (IM)
- For women with no contraindications to use and identified risk factors for PPH having an actively managed third stage

NB: Ergometrine commonly produces nausea and vomiting and may lead to raised BP (The Royal Australian and New Zealand College of Obstetricians and Gynaecologists).

Contradictions to the use of Syntometrine ®:

- Pre-eclampsia or eclampsia.
- Hypertensive disorders.
- No knowledge of the woman's antenatal and medical history and the birth is imminent, including women with precipitous labours in whom blood pressure has not been checked.
- Severe hepatic impairment, renal failure or sepsis - avoid use as sensitivity to ergometrine is increased.
- Severe fibroids.
- Allergy to oxytocin or ergometrine.

NB: Although pre-eclampsia, hypertensive disease in pregnancy, severe fibroids and sepsis are identified risk factors for postpartum haemorrhage, women with these conditions must not be given Syntometrine.

Precautions to the use of Syntometrine

Pre-existing pulmonary, cardiac or vascular disorders: Ischaemic heart disease, peripheral vascular disease or hypertension may be exacerbated, migraine; Raynaud's phenomenon (extreme vasoconstriction of peripheral blood vessels), respiratory disease or chronic anaemia.

7. Physiological management of third stage

Physiological management is suitable for women without risk factors. (Refer to: Appendix C for Third Stage Physiological management exclusion criteria)

A physiological third stage of labour is achieved when the delivery of the placenta and membranes occur spontaneously, and haemostasis is achieved physiologically.

Physiological management of third stage is associated with a higher rate of PPH.

The placenta and membranes are expelled by maternal effort only and without using uterotonic agents or controlled cord traction. The process may take from fifteen minutes to one hour and is individual to the woman.

- No uterotonic drugs are given. A woman may wish to convert to active management at any point to shorten third stage.
- Observation of the woman's clinical obs and blood loss to continue beyond delivery of the placenta.
- Promote skin-to-skin contact between woman and the newborn. Early breast-feeding aids separation and expulsion of the placenta and membranes by natural release of oxytocin.
- Clamp cord after pulsation ceased.
- If the cord requires cutting due to neonatal requirements, clamp and cut the cord as usual process.
- The umbilical cord should be handled as little as possible and pulling of the cord should be avoided. It is an important part of the physiological third stage for baby's blood volume to settle. A taut cord or trauma may lead to spasm and will inhibit this process
- In the event of a PPH, immediate active management is required. Refer to [Postpartum Haemorrhage Guideline](#)
- Do not perform controlled cord traction. Such management when uterotonics have not been administered may lead to partial detachment of the placenta, leading to PPH. There is also a risk of uterine inversion.
- The woman should be allowed to adopt whatever position she wishes. Encouraging women to adopt an upright position shortly after birth, may assist in reducing blood loss without the necessity of resorting to uterotonics and cord traction.
- If the membranes start to tear use a sponge forceps to clamp and twist remaining membranes into a remove any pieces of membrane that are present.
- Document time of birth of placenta and membranes.
- Immediately following removal of the placenta and membranes, massage the fundus of the uterus to make sure it is well contracted

8. Umbilical blood collection and analysis

8.1 Cord blood collection

- Samples can be taken before the umbilical cord is clamped.
- Cord blood should be taken for all babies of Negative and O Positive blood group Mothers.
- Ideally done with minimal delay after birth of neonate to improve the chance of obtaining sample and minimising risk of sample clotting.

- If the cord has been clamped, release clamp on placental (maternal) side of cord and drain sample into supplied galley pot in birth pack (approximately 10 mL of blood).
- Collect immediately into syringe and transfer to relevant pathology tubes. Hand write "cord blood" on tubes until neonatal MRN stickers are available.

8.2 Umbilical cord blood gas samples

Umbilical cord blood gas sampling is the most objective determinant of foetal metabolic condition at the time of birth. Collection and assessment of paired cord bloods samples for analysis are useful in screening for and investigating the origin of intrapartum asphyxia and acidaemia.

- All babies, regardless of mode of delivery, are to have cord blood gas samples collected after birth.
- Paired cord blood samples should be collected from all babies, regardless of mode of delivery.
- Samples can be taken before the umbilical cord is clamped.

9. Retained placenta

Retained placenta is defined as the third stage that has not been completed within 30 minutes of active management or 60 minutes for physiological management following the delivery of the neonate.

Procedure

- Notify the GPO and **the labour ward co-ordinator of a suspected retained placenta** if:
The woman is bleeding (refer to PPH guideline), or
- The placenta has not delivered within 30 minutes of the birth of the baby (active management), or
- The placenta has not delivered within 60 minutes of the birth of the baby (physiological management) – conversion to active management may need to be considered.
- Bladder assessment: Perform bladder catheterisation. A full bladder may interfere with the descent and delivery of the placenta.

Assess for placental separation.

If the placenta is separated:

- Encourage maternal position changes.
- Apply continuous controlled cord traction, with counterpressure to the uterus. Do NOT ask mother to push.
- Offer a vaginal examination to determine if the placenta is trapped in the cervix or lower segment.
- Encourage breastfeeding or nipple stimulation.

Management if the placenta remains retained: Avoid vigorous cord traction to prevent cord snapping or uterine inversion

If bleeding: See Postpartum Complications guideline for management if PPH.

If not bleeding, intravenous oxytocin should not be routinely started.

Collect blood for full blood picture and cross-matching.

- The WHO recommends that in the absence of haemorrhage, the woman should be observed for a further 30 minutes following the initial 30 minutes (a maximum of 1 hour all together) before manual removal of the placenta is attempted and the timing of manual removal as the definitive treatment is left to the judgment of the clinician.
- In the event of a physiological third stage, conversion to active management may need to occur (with consent) if the delay extends beyond one hour with no significant bleeding. Proceed to active management of the third stage and administer Oxytocin 10IU IM.

10. Examination of placenta and membranes

- Following the delivery of the placenta and membranes, a midwife is required to check the placental tissue and membranes.
- Examination includes visualisation and documentation of the two umbilical arteries and an umbilical vein and any variants to this.
- Any variation of the placenta compared to normal placental anatomy should also be documented.
- The status of the placenta and membranes is to be documented on the Labour and Birth Summary and entered in to the STORK data base and verbally handed over via the clinical handover process.
- If there are any concerns that the placenta or membranes are not complete, the on-call GPO must be notified
- Unless the placenta is being sent to pathology, all placentas are to be placed in a plastic bag and discarded into the clinical waste bin.
- Placentas that are to be kept and stored must be labelled with patient sticker, date and time of birth and mode of delivery and placed in the dedicated placenta fridge in the labour ward pan room.
- If the woman wishes to take her placenta home, ensure she completes the Release of Human Tissue and explanted Medical Device Consent AKMR 30.3 and refer to: [Birth-Women Taking Placenta Home Procedure](#). Encourage her to have the placenta taken home ASAP to prevent accidental discarding.

11. Measuring blood loss

- Collect and measure blood loss and add this to the estimated blood loss (i.e. loss that cannot be measured) to obtain total blood loss.
- Document Observations
- Document total blood loss in DMR, STORK, patient care plan and clinical manager.

Refer to the [Spontaneous Labour and Birth Guideline](#) for guidance on clinical observations following labour and birth. (section 9.2)

12. Legislative context

The following documents are required to give effect to this procedure Legislation.

- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP 0095](#)

- Health Department of Western Australia (HDWA) [Recognising and Responding to Acute Deterioration MP 0086/18](#)
- HDWA [Consent to Treatment Policy OD0657/16](#)
- East Metropolitan Health Service (EMHS) Policy
- Armadale Kalamunda Group (AKG) Policy

13. Supporting information

The following documents support the implementation of this procedure:

- [Non-Standard Management Plans: Obstetrics and Gynaecology Procedure](#)
- [Spontaneous Labour and Birth Guideline](#)
- [Clinical Handover Procedure AKG-PRO-0521-19](#)
- [Postpartum Haemorrhage Guideline OBS-GUI-0365-1](#)
- [Post-Partum Urinary Retention Guideline](#)
- [Recognition and Response to Acute Deterioration Policy AKG-POL-0388-14](#)
- [Consent Policy EMHS: 5](#)
- [Birth - Women Taking Placenta Home Procedure](#)

14. Compliance, monitoring and evaluation

Compliance against this procedure will be evaluated by monitoring of reported clinical incidents via Datix CIMS.

Specific findings are to be reported at departmental meetings for review and identification of any opportunities for improvement.

15. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 6 – Action 6.3: Partnering with Consumers
- Standard 6 – Action 6.5: Correct Identification and procedure matching
- Standard 6 – Action 6.7: Clinical Handover
- Standard 6 – Action 6.9: Communicating critical information
- Standard 8 – Action 8.6: Escalating care
- Standard 8 – Action 8.11: Responding to deterioration

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17. Policy owner (relating to these procedures)

HoD Obstetrics and Gynaecology

Enquiries relating to this procedure may be directed to:

Title: A/Associate Midwifery Manager

Department: Obstetrics and Gynaecology

Email: Tracey.Crotty@health.wa.gov.au

18. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V6.0	22/08/2025	22/08/2028	Scheduled review including Grammatical and wording amendments Section 9 - Procedure Notify the GPO and the labour ward co-ordinator of a suspected retained placenta if: The woman is bleeding (refer to PPH guideline), or The placenta has not delivered within 30 minutes of the birth of the baby (active management), or The placenta has not delivered within 60 minutes of the birth of the baby (physiological management) - conversion to active management may need to be considered. Bladder assessment: Perform bladder catheterisation. A full bladder may interfere with the descent and delivery of the placenta. Assess for placental separation. If the placenta is separated: Encourage maternal position changes. Apply continuous controlled cord traction, with counterpressure to the uterus. Do NOT ask mother to push. •Offer a vaginal

			examination to determine if the placenta is trapped in the cervix or lower segment. •Encourage breastfeeding or nipple stimulation. Management if the placenta remains retained: Avoid vigorous cord traction to prevent cord snapping or uterine inversion If bleeding: See Postpartum Complications guideline for management if PPH. If not bleeding, intravenous oxytocin should not be routinely started. Collect blood for full blood picture and cross-matching. Section 10: additional dot point “Placentas that are to be kept and stored must be labelled with patient sticker, date and time of birth and mode of delivery and placed in the dedicated placenta fridge in the labour ward pan room” Section 11: dot point 2 Change to “Document total blood loss in DMR, STORK, patient care plan and clinical manager
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19. Authorisation

This procedure has been authorised and issued by the Director Nursing and Midwifery.

Authorised by	Russell Colyer-Cockburn – A/Director Nursing and Midwifery
Authorisation date	22/08/2025
Published date	22/08/2025

Appendix 1. Third Stage Management

Third Stage: from birth of the baby to birth of the placenta and membranes

Active management (recommended for all births):

- Administration of Oxytocic immediately after the birth of the neonate
- Wait at least 1–3 minutes after birth or for cord pulsation to cease and then clamp and cut cord
- Clamp and cut earlier if concern for neonate or cord integrity
- Do not manipulate or massage the fundus
- Wait for signs of placental separation
- Controlled cord traction (CCT) and uterine guarding after signs of separation.

Physiological management:

- Suitable for well informed women without risk factors
- Placenta birthed by maternal effort/gravity
- Oxytocin not administered
- No manipulation or massage of fundus
- Leave cord unclamped until pulsation has ceased or if cut leave the maternal end unclamped
- Wait for signs of placental separation and use hands-off approach:
 - No controlled cord traction (CCT)
 - Placenta and membranes birth spontaneously and/or with maternal effort
- Ongoing care includes:
 - Encourage upright position
 - Ensure bladder empty
 - Maintain calm, warm and relaxed environment
 - Support privacy and reduce interruptions
 - Encourage to focus on physiological process
 - Observe general physical condition and vital observations
- Prolonged after 60 minutes
- Recommend intervention with oxytocin and revert to active management if bleeding needs to be controlled

Appendix 2. Third Stage Physiological Exclusion Criteria

Exclusion Criteria	• Previous history of primary postpartum haemorrhage. • Abnormal uterine anatomy: e.g. fibroids, uterine septum, previous uterine surgery including caesarean. • Over distended uterus: due for example to; multiple gestation, big baby or polyhydramnios. • Parity of 5 or greater. • Abnormalities of the placenta: e.g. low-lying placenta, placenta previa, vasa praevia • Antepartum haemorrhage. • Haemoglobin (Hb) of less than 110 g/l. • Abnormalities of coagulation: due to for example, foetal death in utero, hypertension, clotting diseases, anti-coagulant therapy. • General infections. • Obstetric or anaesthetic interventions: e.g. induction, augmentation, epidurals, forceps, vacuum, shoulder dystocia, episiotomy or tear requiring immediate suturing. • Intrapartum haemorrhage e.g. active bleeding from a 3rd or 4th degree tear • Uterine muscle exhaustion: due for example to induction, augmentation or labour longer than 18hrs with primigravida or 12hrs with a multigravida or maternal exhaustion. • Intra amniotic infection: as indicted by pyrexia and/or prolonged ruptured membranes (<24 h). • Drug induced uterine hypotonia: e.g. nifedipine and magnesium sulphate and terbutaline
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